

Question:
A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?
Answer Choices:
A. The patient is at risk for double depression.
B. The patient may have symptoms of mania or psychosis.
C. The patient is likely to show anhedonia.
D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

<think>
Finding Reasoning Paths :
1 . The patient's history of feeling sad since childhood is consistent with a long-standing mood disorder , possibly major depressive disorder (MDD), which is more common in older adults .
2 . Recent worsening of symptoms , including lack of hope , energy , and concentration , along with suicidal ideations and feelings of hopelessness , aligns with a depressive episode , likely within the context of MDD .
3 . The patient's feeling that " if something terrible can happen , it usually will " suggests existential anxiety or pessimistic cognitive distortions , which are common in major depressive disorder .
4 . The presence of personality traits such as blame of the self for past problems and difficulty with decision-making further supports the possibility of a major depressive disorder .
5 . The condition's chronicity (since childhood) and recent exacerbation suggest a persistent depressive disorder (PDD) or MDD , but the acute nature of the current episode (worsening symptoms over the past 2 weeks) makes MDD more likely .

Reasoning Process :
1 . **Initial Presentation**: The patient reports feeling sad since childhood , which is significant because it implies a long history of mood disturbances . This is a key feature of major depressive disorder (MDD) or persistent depressive disorder (PDD), both of which are more commonly seen in older adults .

2 . **Recent Worsening of Symptoms**: The patient's recent worsening symptoms include :
- Lack of hope .
- Severe energy deficit and difficulty with concentration .
- Suicidal ideations .
- Persistent feelings of hopelessness .
These symptoms are characteristic of a depressive episode within MDD , as the current episode appears to be superimposed on a chronic mood disorder .

3 . **Suicidal Ideations**: Suicidal ideations are a hallmark of major depressive episodes and are highly concerning . The patient's statement that suicidal thoughts are worsening suggests a severe depressive episode and underscores the need for careful evaluation and intervention to ensure safety .

4 . **Cognitive Symptoms**: The patient mentions difficulty making decisions and feeling hopeless . These cognitive symptoms are consistent with a major depressive episode , as depression often impairs executive functioning and decision-making abilities .

5 . **Personality Traits**: The patient's self-blame for past problems and decision-making difficulties could be indicative of depressive ideation or generalized anxiety . However , the chronicity of these traits since childhood is more consistent with a persistent mood disorder , such as MDD , rather than a transitory condition .

6 . **Risk of Double Depression**: The patient's history of a long-standing mood disorder and the worsening of recent symptoms raises the possibility of a " double depression " pattern . In this scenario , the patient has a pre-existing mood disorder (likely MDD or PDD) that has worsened due to a current episode . This is a common pattern in older adults .

7 . **Exclusion of Other Conditions**: The lack of manic or psychotic symptoms makes bipolar disorder or psychotic depression less likely . Similarly , the chronicity and age of onset make schizophrenia or schizoaffective disorder less probable . Obsessive-compulsive disorder (OCD) or other anxiety disorders are also unlikely , given the focus on mood and depressive symptoms .

Conclusion :
The most likely condition for this patient is major depressive disorder , with the possibility of a " double depression " pattern due to the long-standing history of a mood disorder and the recent worsening of symptoms . The patient is at high risk for recurrent depressive episodes and should be closely monitored for future episodes .
</think>
<answer>
The patient is at risk for double depression .
</answer>